9-B, Bhawanipur Colony,
Annpurna Main Road, Indore
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Patient ID	: 290324052	Registered On	: 29-Mar-2024 05:30 PM
Patient Name	: **MR. ASHOK 5**	Sample Collected On	: 29-Mar-2024 05:45 PM
Age/Gender	: 31 Yrs / Male	Sample Reported On	: 01-Apr-2024 11:55 AM
Ref By	: DR. SELF	Sample ID	*290324052*
		Payment Status	: **Due**

HAEMATOLOGY

Test Name	Observed Values	Unit	Bio Ref. Interval
COMPLETE BLOOD COUNT (CBC - 5 Part)			
Hemoglobin	14.0	g/dl	13.5-17.5
R.B.C. Count	5.36	milli./cu.mm	4.5-5.9
Packed Cell Volume	44.1	%	37-53
Mean Cell Volume(MCV)	82.3	fl	80-100
Mean Cell Hemoglobin(MCH)	26.1	pg	26-34
Mean Cell Hb Conc(MCHC)	**31.7**	g/dl	32-36
RDW (CV)	13.2	%	11 - 16
RDW (SD)	42.0	fl	35 - 56
Total W.B.C Count	8060	/cumm	4500-11000
Differential Count			
Neutrophils	54	%	35-75
Lymphocytes	35	%	24-44
Monocytes	09	%	2-12
Eosinophils	02	%	0-6
Basophils	00	%	0-1
Absolute Differential Count :			
Absolute Neutrophils Count	4352	/ cumm	1575 - 8250
Absolute Lymphocytes Count	2821	/ cumm	1080-4840
Absolute Monocytes Count	725	/ cumm	90-1320
Absolute Eosinophils Count, AEC	161	/ cumm	0 - 660
Neutrophil-to-lymphocyte ratio, NLR	1.5	%	0.78 - 3.53
Platelet Count	2.66	Lakh/cumm	1.5-4.5
Mean Platelet Volume (MPV)	11.2	fl	6.5 - 12.0
Platelet Distribution Width (PDW)	16.4		15-17

Sample Type: EDTA Whole blood
Performed on:- "Mindray BC-6200" Fully Automated 5 Part Hematology Analyzer.
Methods: - Hemoglobin by colorimetric method. Total RBC count, platelets count by electrical Impedance method. PCV, MCV, MCH, MCHC: calculated, RDW-CV by RBC Histogram, WBC Parameters by Laser scatter fluorescence flow cytometry/microscopy, MPV, PDW by platelet Histogram IPF by PLTO scattergram. Reference: - Nathan DG,and Oski,FA(1981)Hematology of Infancy and Childhood, ed 2, WB Saunders, pp1552. Dallman PR (1977) pediatrics, 16th edition, Appleton-Century-Crofts, p 1111 pediatrics Normal Range (1995) Children's Hospital of Buffalo, Coulter Viewpoint, No.17. p 8 Pediatrics Normal Range Study (1995) Children's Hospital, Minneapolis and St. PAUL.

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Patient ID	: 290324052	Registered On	: 29-Mar-2024 05:30 PM
Patient Name	: **MR. ASHOK 5**	Sample Collected On	: 29-Mar-2024 05:45 PM
Age/Gender	: 31 Yrs / Male	Sample Reported On	: 01-Apr-2024 11:56 AM
Ref By	: DR. SELF	Sample ID	* 2 9 0 3 2 4 0 5 2 *
		Payment Status	: **Due**

Blood Group

Test Name	Observed Values	Unit	Bio Ref. Interval
ABO Group	**"A" Rh Positive**		

Sample Type : EDTA Whole Blood
Method : Slide Agglutination Test
Limitations :The test is accurate and will detect the common blood grouping system A,B,O,AB and Rhesus(D). Unusual blood groups or rare sub-types will not be detected by this method. Further investigation by a blood transfusion laboratory will be necessary to identify such groups.

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BIOCHEMISTRY

Test Name	Observed Values	Unit	Bio Ref. Interval
Creatinine	0.93	mg/dL	0.5-1.5
GFR	101	ML/Min/1.73M	>60

Sample Type : Serum
Method :- CKD-EPI Creatinine Equation.
Interpretation :Clinical Significance :-The normal serum creatinine reference interval dose not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone.Thus,it is recommended for clinical laboratories to routinely estimate glomerular filtration rate(eGFR), a "gold standard" measurement for assessment of renal function and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible.It cannot be measured easily in clinical practice,instead,GFR is estimated from equations using serum creatinine, age , race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Appliction of CKD- EPI equation together with the other diagnostic tools in renal medicine will further imporve the detection and mangement of patients with CKD.
Reference : Levey AS, Stevens LA, Schmid CH,Zhang YL,Castro AF,3rd,Feldman HI, et al. A new equation to estimate glomerular filtration rate, Ann Intern Med. 2009;150(9):604-12. Please correlate with clinical conditions.

SGPT (ALT)	18	U/L	0 - 42

Sample Type : Serum
Method: UV-assay accordingto IFCC

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REPORT

Pathology • Sonography • Digital X-Ray • Colour Doppler • ECG • Echocardiography • TMT

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Urine Routine Examination

Test Name	Observed Values	Unit	Bio Ref. Interval
Physical Examination			
Quantity	30 ML		
Colour	Pale Yellow		Pale Yellow
Appearance	Clear		Clear
Reaction(pH)	6.5		6.0 - 8.5
Specific Gravity	1.015		1.005 - 1.030
Chemical Examination			
Protein	Absent		Absent
Glucose	Absent		Absent
Ketones	Absent		Absent
Bile Salt / Bile Pigment	Negative		Absent
Occult Blood	Negative		Absent
Urobilinogen	Negative		Absent
Nitrite	Absent		Absent
Microscopic Examination			
Pus Cells	2-3	/ HPF	0-5
Epithelial cells	3-4	/ HPF	
Red Blood Cells	Nil	/ HPF	Nil
Spermatzoa	Absent		Absent
Casts	Absent		Absent
Crystals	Absent		Absent
Yeast Cell	Absent		Absent
Bacteria	Absent		Absent

Sample Type : Urine

Method: Urinalysis-Multiple Reagent Strip Method & Microscopy done after centrifugation of 2500 rpm for 5 minutes. Glucose oxidase-peroxidase, protein - Tetrabromophenol blue, Ketone-Sodium nitroprusside, Blood-Peroxidase activity of hemoglobin, Bilirubin-Diazotized dichloroaniline, Urobilinogen - Ehrlich Method.

Please look for drug history (SGLT-2 inhibitors) in diabetic patient if urine sugar is irrational high.

---------- **END OF REPORT** ----------

Dr.Vidhi Rathi Maheshwari
MBBS,M.D
Lab Director, Pathologist

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